

IBUX® TABLET 400 mg

Ibuprofen Tablet 400 mg

NAME AND STRENGTH OF ACTIVE SUBSTANCE

Each tablet contains:

Ibuprofen 400 mg

PRODUCT DESCRIPTION

Ibux® Tablet 400 mg is Yellow film coated, round convex, 13 mm diameter impressed with 'SL' logo on one side.

Colouring agent: Quinoline Yellow Al-lake, titanium dioxide and iron oxide yellow.

PHARMACODYNAMICS

Ibuprofen is a propionic acid derivative with analgesic, anti-inflammatory and anti-pyretic activity. The drugs therapeutic effects as an NSAID are thought to result from its inhibitory effect on the enzyme cyclo-oxygenase, which results in a marked reduction in prostaglandin synthesis. Ibuprofen is rapidly absorbed from the gastrointestinal tract, peak serum concentrations occurring 1-2 hours after administration. The elimination half-life is approximately 2 hours.

PHARMACOKINETICS

Ibuprofen is metabolised in the liver to two inactive metabolites and these together with unchanged ibuprofen, are excreted by the kidney either as such or as conjugates. Excretion by the kidney is both rapid and complete. Ibuprofen is extensively bound to plasma proteins.

INDICATION

Ibux® Tablet 400 mg relieves pain such as dysmenorrhoea, rheumatic pain and muscular pain. Ibux® also relieves inflammation of muscles and joints in rheumatic disease including juvenile arthritis and other muscular skeletal disorder.

RECOMMENDED DOSAGE

Adults and children aged 12 years and above: Initial dose is 400 mg, then if necessary, 400 mg-800 mg every 4 hours. It is recommended not to exceed 1200 mg daily for minor ailments. It can be used up to a maximum 2400 mg daily, if prescribed by a doctor. After assessing the risk/benefit ratio in each individual patient, the lowest effective dose for the shortest possible duration should be used. Ibux® should be taken with or after food.

ROUTE OF ADMINISTRATION

Oral

CONTRAINDICATIONS

Active peptic ulceration and gastro-intestinal bleeding. Known hypersensitivity to ibuprofen. Not to be given when acetylsalicylic acid or other non-steroidal anti-inflammatory drugs have caused asthma, urticaria or rhinitis.

WARNINGS AND PRECAUTIONS

Caution is required if Ibux® Tablet 400 mg is administered to patients suffering from or with a previous history of bronchial asthma since ibuprofen has been reported to cause bronchospasm in such patients. Ibux® Tablet 400 mg should only be given with care to patients with a history of gastrointestinal disease.

Caution is required in patients with renal, hepatic or cardiac impairment since the use of NSAIDs may result in deterioration of renal function and should be monitored in these patients.

Ibux® Tablet 400 mg should be given with care to patients with a history of heart failure or hypertension since oedema has been reported in association with ibuprofen administration.

Cardiovascular Thrombotic Events

Observational studies have indicated that non-selective NSAIDs may be associated with an increased risk of serious cardiovascular events, principally myocardial infarction, which may increase with dose or duration of use. Patients with cardiovascular disease or cardiovascular risk of an adverse cardiovascular event in patient taking NSAIDs, especially in those with cardiovascular risk factors, the lowest effective dose should be used for the shortest possible duration. There is no consistent evidence that the concurrent use of aspirin mitigates the possible increased risk of serious cardiovascular thrombotic events associated with NSAIDs use.

Hypertension

NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking anti-hypertensive with NSAIDs may have an impaired anti-hypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAID treatment and at regular intervals thereafter.

Heart Failure

Fluid retention and oedema have been observed in some patients taking NSAIDs, therefore caution is advised in patients with fluid retention or heart failure.

Gastrointestinal Events

All NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation which may increase with dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g. the elderly, those with a history of serious gastrointestinal events, smoking

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and alcoholism. When gastrointestinal bleeding or ulcerations occur in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patients about signs and symptoms of serious gastrointestinal adverse events.

Severe Skin Reactions

NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reactions and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.

RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAID

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without symptoms, in patients treated with NSAID therapy. Although minor GI problems (eg. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (eg. alcoholism, smoking, corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

INTERACTIONS WITH OTHER MEDICAMENTS

Care should be taken in patients treated with any of the following drugs as interactions have been reported in some patients.

Anti-hypertensive, anti-diuretic, cardiac glycosides, Lithium, Methotrexate, cyclosporin, mifepristone and other analgesic.

STATEMENT ON USAGE DURING PREGNANCY AND LACTATION

Use in Pregnancy: Ibuprofen is not recommended during pregnancy unless in the judgement of the physician the potential benefits outweigh the possible hazards to the foetus.

Use in Lactation: Ibuprofen is not recommended during lactation unless the expected benefits to the mother outweigh the potential risk to the neonate.

ADVERSE EFFECTS/ UNDESIRABLE EFFECTS

Gastro-intestinal side effect such as epigastric pain, nausea, heartburn, abdominal discomfort, and sensation of "fullness" in gastro-intestinal tract are frequent. Other side effects include headache, dizziness, skin rashes, pruritus, tinnitus, oedema, depression, drowsiness, insomnia, blurred vision and other ocular reaction.

Hypersensitivity reactions, abnormalities of liver function tests, impairment of renal function, agranulocytosis and thrombocytopenia have occasionally been observed.

Peptic ulceration and gastro-intestinal bleeding have been reported.

OVERDOSE AND TREATMENT

Symptoms include nausea, vomiting, dizziness and rarely, loss of consciousness. Large overdoses are generally well tolerated when no other drugs are involved.

Treatment is symptomatic and includes stomach lavage. There is no specific antidote to ibuprofen.

STORAGE CONDITION

Store in a cool dry place at below 30°C, protect from direct light and humidity. The shelf-life of Ibux[®] Tablet 400mg is 3 years.

DOSAGE FORMS AND PACKAGING AVAILABLE

Blister of 50 x 10's tablet.

MANUFACTURER

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PRODUCT REGISTRATION HOLDER

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DATE OF REVISION OF PACKAGE INSERT

PM0037/14-12-18/REV05